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call for earlier attention to drugs that have promise in treating such conditions, including early consultation with FDA for sponsors of such products and efficient trial design, potentially relying on well-controlled phase 2 studies for evidence of effectiveness. The subpart E regulations specifically recognize that patients and physicians are generally willing to accept greater risks and side effects from treatment of life-threatening and severely debilitating diseases than they would for other diseases. The four principal programs that support these principles are fast track designation, breakthrough therapy designation, accelerated approval, and priority review designation (referred to in this guidance as the Agency's *expedited programs*).

FDA has a history of applying the philosophy underlying subpart E to drugs for rare diseases through use of the Agency's expedited programs. FDA recognizes that certain aspects of drug development that are feasible for common diseases may not be feasible for rare diseases and that development challenges are often greater with increasing rarity of the disease. FDA will continue to apply flexibility in these situations to address particular challenges posed by each disease.

III. CONCEPTS FOR EXPEDITED PROGRAMS

The programs that are the subject of this guidance, fast track designation, breakthrough therapy designation, accelerated approval, and priority review, are summarized in [section IV](#) and described individually in detail in sections [V](#), [VI](#), [VII](#), and [VIII](#). All four expedited programs represent efforts to address an unmet medical need in the treatment of a serious condition, which is discussed in the following paragraphs.

A. Serious Condition

1. Whether a Condition Is Serious

FDA intends to interpret the term *serious* as it has done in the past for the purposes of accelerated approval⁵ and expanded access to investigational drugs for treatment use.⁶ A serious disease or condition is defined in the expanded access regulations as follows:

... a disease or condition associated with morbidity that has substantial impact on day-to-day functioning. Short-lived and self-limiting morbidity will usually not be sufficient, but the morbidity need not be irreversible if it is persistent or recurrent. Whether a disease or condition is serious is a matter of clinical judgment, based on its impact on such factors as survival, day-to-day functioning, or the likelihood that the disease, if left untreated, will progress from a less severe condition to a more serious one.⁷

⁵ Food and Drug Administration, Final Rule, New Drug, Antibiotic, and Biological Drug Product Regulations; Accelerated Approval (57 FR 58942, December 11, 1992) and Food and Drug Administration, Proposed Rule, New Drug, Antibiotic, and Biological Drug Product Regulations; Accelerated Approval (57 FR 13234, April 15, 1992).

⁶ Part 312, subpart I.

⁷ 21 CFR 312.300(b)(1).

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Note: For the purposes of this guidance, the terms *condition*, *disease*, and *illness* are used interchangeably. All conditions meeting the definition of life-threatening as set forth at § 312.81(a) would also be serious conditions.

2. *Whether the Drug Is Intended to Treat a Serious Condition*

As referenced in [section IV](#), the statutory and regulatory eligibility criteria for expedited programs require that a drug be intended to treat a serious condition. To satisfy this criterion, a drug must be intended to have an effect on a serious condition or a serious aspect of a condition, such as a direct effect on a serious manifestation or symptom of a condition or other intended effects, including the following:

- A diagnostic product intended to improve diagnosis or detection of a serious condition in a way that would lead to improved outcomes
- A product intended to mitigate or prevent a serious treatment-related side effect (e.g., serious infections in patients receiving immunosuppressive therapy)
- A product intended to avoid or diminish a serious adverse event associated with available therapy for a serious condition (e.g., product that is less cardiotoxic than available cancer therapy)⁸
- A product intended to prevent a serious condition or reduce the likelihood that the condition will progress to a more serious condition or a more advanced stage of disease

B. Available Therapy

For purposes of this guidance, FDA generally considers *available therapy* (and the terms *existing treatment* and *existing therapy*) as a therapy that:

- Is approved or licensed in the United States for the same indication being considered for the new drug⁹ and
- Is relevant to current U.S. standard of care (SOC) for the indication

⁸ Sponsors considering an expedited drug development designation or program for a drug intended to avoid a serious adverse event associated with available therapy or diminish its severity should be aware that they will need to provide data that directly support the effect corresponding to the level of evidence needed to meet the qualifying criteria for the relevant designation or program (e.g., phase 3 data demonstrating lower incidence or severity of the serious adverse reaction compared to available therapy for priority review). The requisite data may be very difficult to obtain in early development, particularly for purposes of breakthrough therapy designation.

⁹ There may be a substantial number of approved therapies with varying relevance to how a serious disease is currently treated in the United States, including therapies that are no longer used or are used rarely. Only in exceptional cases will a treatment that is not approved for the indicated use or is not FDA-regulated (e.g., surgery) be considered available therapy. In those cases, FDA may consider an unapproved or unlicensed therapy to constitute available therapy if the safety and effectiveness of the use is supported by compelling evidence, including extensive evidence in the published literature (e.g., certain well-established oncologic treatments).

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FDA's available therapy determination generally focuses on treatment options that reflect the current SOC for the specific indication (including the disease stage) for which a product is being developed. In evaluating the current SOC, FDA considers recommendations by authoritative scientific bodies (e.g., National Comprehensive Cancer Network, American Academy of Neurology) based on clinical evidence and other reliable information that reflects current clinical practice. When a drug development program targets a subset of a broader disease population (e.g., a subset identified by a genetic mutation), the SOC for the broader population, if there is one, generally is considered available therapy for the subset, unless there is evidence that the SOC is less effective in the subset.

Over the course of new drug development, it is foreseeable that the SOC for a given condition may evolve (e.g., because of approval of a new therapy or new information about available therapies). FDA will determine what constitutes available therapy at the time of the relevant regulatory decision for each expedited program a sponsor intends to use (e.g., generally early in development for fast track and breakthrough therapy designations, at time of biologics license application (BLA) or new drug application (NDA) submissions for priority review designation, during BLA or NDA review for accelerated approval). FDA encourages sponsors to discuss available therapy considerations with the Agency during interactions with FDA.

As appropriate, FDA may consult with special Government employees or other experts when making an available therapy determination.

When determining whether a drug granted accelerated approval or approved with a risk evaluation and mitigation strategy (REMS) that includes elements to assure safe use (ETASU) under section 505-1 of the Federal Food, Drug, and Cosmetic Act (FD&C Act) (21 U.S.C. 355-1), is considered available therapy, the following principles will be applied:

- A drug would not be considered available therapy if the drug is granted accelerated approval based on a surrogate endpoint or an intermediate clinical endpoint and clinical benefit has not been verified by postapproval studies. (See [section III.C.3.](#))
- A drug would be considered available therapy if the drug is granted accelerated approval because of restricted distribution and the study population for the new drug under development is eligible to receive the approved drug under the restricted distribution program. Similarly, a drug would be considered available therapy if the study population for the new drug under development is eligible to receive the approved drug under the ETASU REMS.

C. Unmet Medical Need

An unmet medical need is a condition whose treatment or diagnosis is not addressed adequately by available therapy. An unmet medical need includes an immediate need for a defined population (i.e., to treat a serious condition with no or limited treatment) or a longer-term need for society (e.g., to address the development of resistance to antibacterial drugs).

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1. Where There Is No Available Therapy

If there is no available therapy for a serious condition, there is clearly an unmet medical need.

2. Where There Is Available Therapy

When available therapy exists for a condition, a new treatment generally would be considered to address an unmet medical need if the treatment:

- Has an effect on a serious outcome of the condition that is not known to be influenced by available therapy (e.g., progressive disability or disease progression when the available therapy has shown an effect on symptoms, but has not shown an effect on progressive disability or disease progression)
- Has an improved effect on a serious outcome(s) of the condition compared with available therapy (e.g., superiority of the new drug to available therapy when either used alone or in combination with available therapy (i.e., as demonstrated in an add-on study))
- Has an effect on a serious outcome of the condition in patients who are unable to tolerate or failed to respond to available therapy
- Can be used effectively with other critical agents that cannot be combined with available therapy
- Provides efficacy comparable to those of available therapy, while (1) avoiding serious toxicity that occurs with available therapy, (2) avoiding less serious toxicity that is common and causes discontinuation of treatment of a serious condition, or (3) reducing the potential for harmful drug interactions
- Provides safety and efficacy comparable to those of available therapy but has a documented benefit, such as improved compliance, that is expected to lead to an improvement in serious outcomes
- Addresses an emerging or anticipated public health need, such as a drug shortage

In some disease settings, a drug that is not shown to provide a direct efficacy or safety advantage over available therapy may nonetheless provide an advantage that would be of sufficient public health benefit to qualify as meeting an unmet medical need. For example, in a condition for which there are approved therapies that have a modest response rate or significant heterogeneity in response, a drug with a novel mechanism of action (but comparable safety and effectiveness) could have the potential to provide an advantage over available therapy in some patients. In such a case, the novel mechanism of action should have a well-understood relationship to the disease pathophysiology. In addition, there should be a reasonable basis for concluding that a significant number of patients may respond differently to the new drug compared with available therapy. Thus, mechanistic diversity, even without a documented efficacy or safety advantage, could be advantageous in disease settings in which drugs become less effective or ineffective over time.

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For example, infectious disease drugs or targeted cancer therapies with novel mechanisms of action, although appearing to have efficacy similar to available therapy across the disease population, could benefit patients who no longer respond to available therapy. Accordingly, FDA intends to consider a range of potential advantages over available therapy beyond those shown in head-to-head comparisons.

3. Where the Only Available Therapy Was Approved Under the Accelerated Approval Program Based on a Surrogate Endpoint or an Intermediate Clinical Endpoint and Clinical Benefit Has Not Yet Been Verified

As discussed in [sections VII](#) and [III.B.](#), FDA recognizes, as a general matter, that it is preferable to have more than one treatment approved under the accelerated approval provisions because of the possibility that clinical benefit may not be verified in postapproval confirmatory trials. FDA will therefore consider products as addressing an unmet medical need if the only approved treatments were granted accelerated approval based on a surrogate endpoint or an intermediate clinical endpoint and clinical benefit has not been verified by postapproval studies.